

Clubfoot Burden
& Treatment
Access in South-
South Region,
Nigeria

Total Reported Cases

495

States Covered

6

Treatment Centres

9

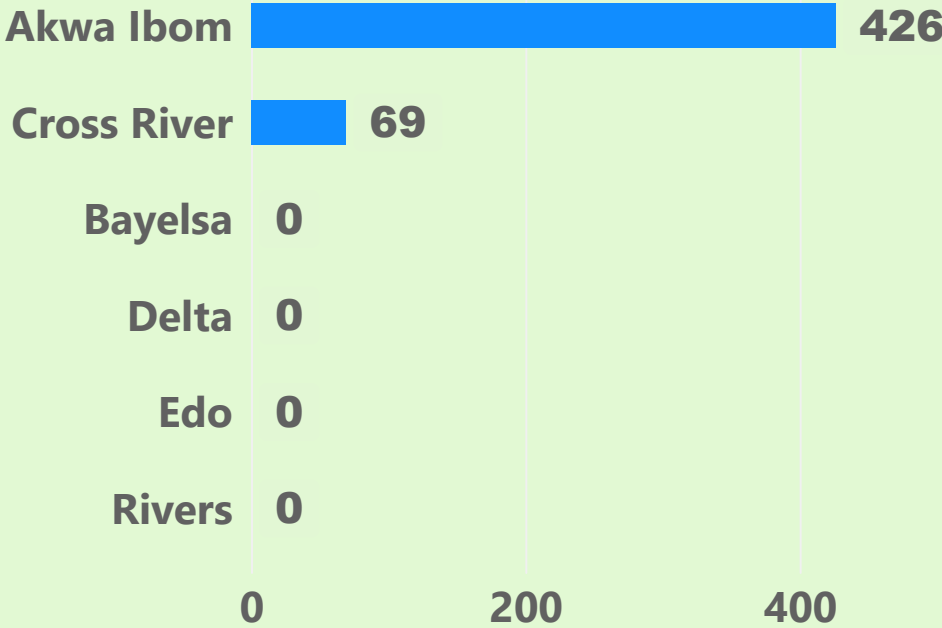
Years Covered

10

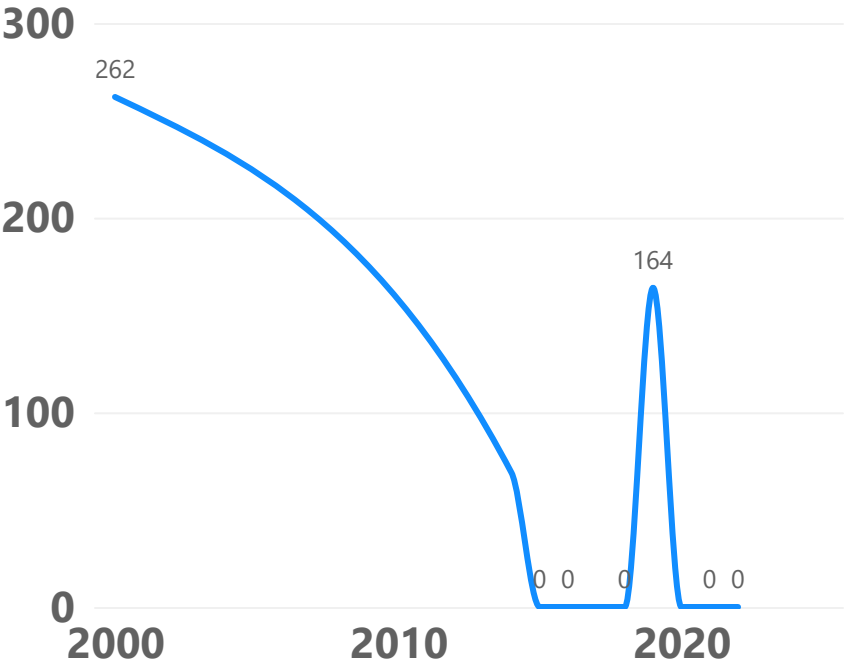
EXECUTIVE

INSIGHT

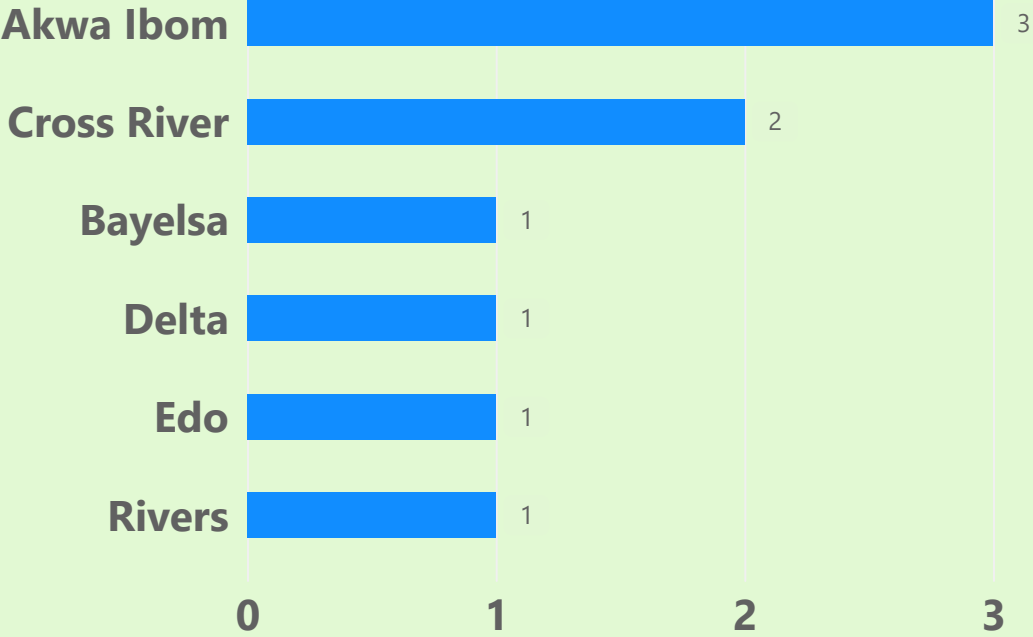
Reported cases by state



Reported cases over time



Treatment Centres by States



Year

2000

2014

2015

2016

2017

2018

2019

2020

2021

State

Akwa I...

Bayelsa

Cross ...

Delta

Edo

Rivers

Data Limitations: Prevalence and estimated expected cases could not be directly calculated because the required population/birth-count and clubfoot incidence data were not available in the private dataset



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Key Insights & Findings

Analysis of reported clubfoot cases and Treatment-centre distribution across six states

EXECUTIVE

INSIGHT



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TOTAL REPORTED CASES

495

CASE CONCENTRATION

86.06%

REPORT CASE COVERAGE

2 of 6

TREATMENT CENTRES

9

BURDEN AND SERVICE DISTRIBUTION

Akwa Ibom has the largest reported case burden and also has the highest number of identified treatment centres in the dataset. Cross River has the second-highest reported case count and the second-highest number of treatment centres.

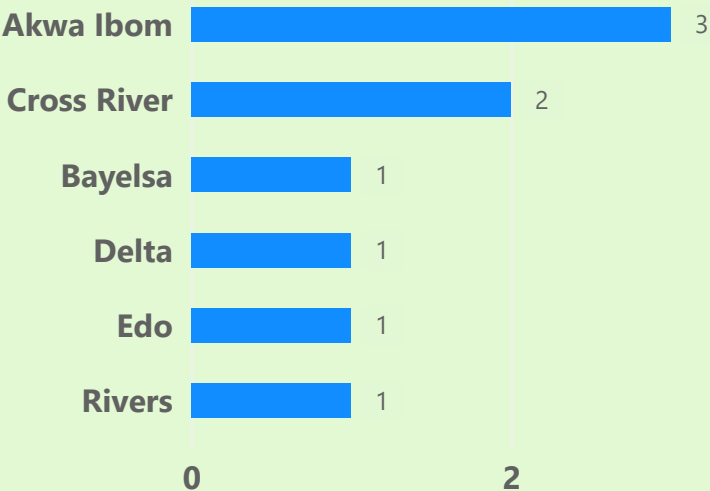
However, reported cases alone cannot establish whether treatment capacity is sufficient because estimated annual need, prevalence and population-adjusted rates are available in the current private dataset.

TREATMENT ACCESS

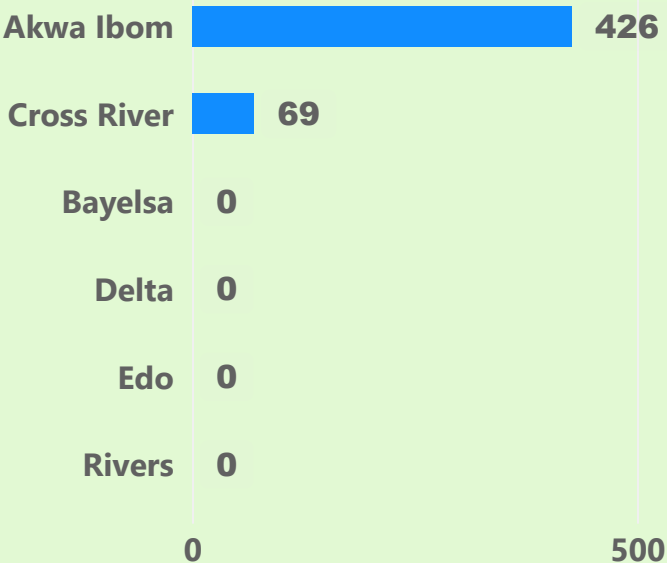
Treatment-Centre availability varies across the six states represented in the dataset. Akwa Ibom has the highest number of identified treatment centres, followed by Cross River, while Bayelsa, Delta, Edo and Rivers have fewer identified centres.

This indicates that treatment services are not evenly distributed geographically across the states.

Treatment Centres by States



Reported cases by state



CASE CONCENTRATION

86.06% of reported cases are concentrated in Akwa Ibom, accounting for 426 of the 495 reported cases in the dataset

This indicates a strong concentration of reported cases in Akwa Ibom within the available dataset

DATA LIMITATIONS

Prevalence/rate and estimated expected cases could not be directly calculated because the required population/birth=count and clubfoot incidence data were not available in the private dataset. Therefore, reported cases are interpreted as observed/reported burden and should not be treated as population-prevalence.

PREVALENCE ANALYSIS- DATA GAP

A state-level prevalence ranking could not be established from the available dataset because a valid population denominator and/or clubfoot prevalence measure was not available.

The reported-case ranking should therefore not be interpreted as a prevalence ranking.

EXPECTED ANNUAL CASES - DATA GAP

Estimated annual clubfoot cases could not be calculated from the available dataset because actual birth counts and an appropriate clubfoot incidence estimate were not available.

Consequently, states cannot be reliably ranked by estimated annual need using the current dataset.